

XXX Inc. Consulting Report
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GSPIA Non-Profit Clinic
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1. Executive Summary

The purpose of this report is to provide XXX Inc. with recommendations informed by historical overview, outcome studies, and the current trends of the therapeutic community industry. Above all, this paper will emphasize how the history of the Therapeutic Community can inform the current and future practices of XXX Inc to build their resilience. Our recommendations are consistent with the standards provided by world and regional Therapeutic Community oversight committees and emphasize making changes that contribute to greater client outcomes as identified by academic outcome studies. Lastly, we offer recommendations for the general portrayal of XXX Inc.'s public marketing & identify future opportunities.

Our Recommendations:

1. Emphasize self-learning by monitoring clients during and after program completion.

The Therapeutic Community's history is complex and was developed in an evolutionary manner. Most early TCs were founded by former graduates looking to help others achieve substance abstinence. As a result, practices have been developed incrementally and inconsistently, with the term 'Therapeutic Community' varying greatly in its meaning between organizations. For the purposes of this paper, we will explore the broadest definition of Therapeutic Communities to be more inclusive to innovative ideas. Therefore, we define the Therapeutic Community as "an organization which uses community-as-a-method to generate personal growth by changing individual behaviors and attitudes."

Because the Therapeutic Community has developed sporadically and incrementally, there has been minimal adherence to overarching standards. As a result, TCs have historically emphasized self-learning and ongoing community participation. A common trend is for the TC to employ its own graduates as a means of ensuring their stability and to serve as role models for current clients. This employment also offers an informed approach as to what worked and did not work from those who have personally experienced the program. Additionally, because TCs are often founded by former graduates, they would bring with them their own experiences. The concept of self-learning and adaptation is fundamental to the TC model.

While the medical community has moved to a results-based & value payment processes, TCs face mounting pressure to document their successful outcomes. *Our research has not revealed consistent indicators for which TCs will be measured.* There has been very little emphasis on holistic measures including trauma scores, and inconsistent outcome measures. Therefore, we instead emphasize the monitoring of program participants and graduates to promote self-learning. XXX's data collection is currently sparse, specifically post-treatment. Therefore, it will be difficult to identify the true impact of program changes. But understanding why people leave treatment early and how treatment has impacted their behaviors is important in verifying your ability to generate personal change.

2. Understand and prioritize the indicators for client success.

After dissecting TCs by specialty (prison-based, faith-based, juvenile-based, etc.) it becomes clear that there cannot be a magic number for the expected success rates of TCs. Client demographics are a large determinant in program outcome. However, research has shown that there are numerous predictors of client success throughout the treatment process, regardless of TC populations served. Important indicators include: treatment length, receiving acknowledgement of their graduation, securing post-treatment employment & housing, and voluntary participation in aftercare (these are detailed further in the report). As an example of possible informed interventions, many TCs have implemented temporary Methadone treatment

to encourage individuals to remain in treatment for longer periods. Even though this intervention runs contrary to the TC's community-as-a-method model, encouraging treatment retention is a research-informed intervention. Understanding the indicators of client success should be a large determinant in shaping the program's methods. Additionally, the implementation of internal monitoring can ensure these interventions are indeed positive for XXX, specifically.

3. Anticipate future trends and funding difficulties and set a clear direction for the type of TC you wish to become.

Historically, TCs suffered greatly during the 2008 recession leading to a large restructuring - specifically across Europe. The breadth of researchable information during this period was much larger across Europe, but is consistent in smaller case studies seen in the US. The current pandemic and financial crisis could further inhibit funding opportunities while also straining the current resources should Wine & Liquor stores remain closed for an extended period of time. The most notable changes across Europe after the 2008 crisis were an increase in specialization and decreased length of stay. TCs adapted to become specialized communities to allow for greater funding partnerships, such as faith-based TCs partnering with churches.

In reality, these trends trace back to before the 2008 crisis, but were expedited over this period. Therefore, we recommend XXX examine how it would operate if the length of stays paid by insurance continue to diminish. As a possibility, one study found negligible differences in a 10-month residential treatment combined with 2 months of day treatment versus a 6-month residential treatment combined with 6 months of day treatment. Therefore, there are potential cost-saving measures that may not sacrifice outcomes.

Lastly, we suggest that XXX think directly about the population it serves best and will continue to serve. This could result in the creation of a more specialized TC, directed toward judicially afflicted individuals. A TC emphasizing judicially afflicted individuals may choose to form partnerships with local correctional facilities. Alternatively, XXX may become a generalist TC, serving a wide variety of clients who cannot find treatment elsewhere. However, identifying your strengths and weaknesses in terms of the population you serve may better inform this decision.

4. Develop a clear and succinct marketing message to the public that describes your interventions and purpose.

The TC's history is incredibly complex. The most famous Therapeutic Community, Synanon, became a cult that greatly tarnished the name of TCs as a process for habilitation. TCs remain shrouded in mystery, often confused with faith-oriented treatment or 12 step processes. Therefore, in the era of specialized TCs it is essential to be very clear about your interventions and your purpose in order to be better understood by the public. While we understand XXX is working on the creation of a website where it can better market itself to the public, it is imperative that they know who they are and how to explain themselves to the public. Our recommendation is to create a basic, clear, and direct statement of purpose and action that can be easily understood by uninformed individuals. We emphasize the simplification of the language, interventions, and goals for uninformed individuals.

5. Find a partner to compare practices with.

Strong partnerships enable information sharing and create resiliency. We recommend finding a willing partner in the field of Therapeutic Communities or to find a like-minded regional oversight committee and work to adhere to their predetermined standards (we recommend using international organizations as the U.S. does not consistently update their standards). XXX's partnership with the Non-Profit Clinic highlights their desire to learn more

about the field, best practices, and innovations. The formation of partnerships with other TCs could benefit XXX through the ability to share information and knowledge, as well as successes and failures. Even more, as we approach a time of uncertainty in the current COVID-19 pandemic, it may be beneficial to have an ally to compare approaches and strategies with.

6. Develop a Strategic HR Plan to Hire and Share Information between Departments.

Internal organization management tools, the use of the ATCA standards (Australian Therapeutic Communities Association), and our team's own observations have consistently revealed a desire for a culture that promotes learning. As the Therapeutic Community continues to move towards a professional staff, professional development and strategic hiring will continue to be imperative. However, professionalization must be balanced with the TC tradition of hiring within their own graduate cohorts. Lastly, there is a clear desire for knowledge among many members of the organization. Information sharing between departments and services within XXX Inc. may work to strengthen the overall TC as well as clarify individuals' roles.

Conclusion:

In conclusion, this executive summary delineates our recommendations for XXX Inc. and has been informed by our academic research and the historical factors impacting Therapeutic Communities. This report emphasizes the research and lessons drawn from international Therapeutic Communities that, we believe, gives a more holistic approach to the Therapeutic Community model. We feel the recommendations will be beneficial for the participants of XXX's TC program and contribute to their viability as a non-profit in an ever-evolving business model. Our primary recommendation is to focus on the results and the aftercare of XXX's clients to ensure best practices. The lack of accurate information is consistent across different TCs and not exclusive to XXX, but program changes should either be informed by data, or be evaluated with data to ensure a positive change. We feel these recommendations will help XXX continue its service to Pittsburgh now and in the future.

2. History & Development of the Therapeutic Community

The field of the TC is important to understand as it details how the field has developed, and how it has adapted to changes in the world. Furthermore, it shows that despite organizations evolving largely independent from each other, they used similar strategies to adapt to changes. Even today, we can see how their changes trend similarly. (See A5)

The Therapeutic Community model is often attributed to the 1958 Synanon Village and founder Charles Dederich, which began as a community for those suffering from addiction to prepare themselves for reintegration back into society. But, in the 1970's, Charles Dederich used his authoritarian grip over the community to close the community and remove reintegration as the primary goal, leading to the transformation into a cult-style community. However, while this infamous failure has led to many misperceptions about TCs, the TC's origins trace back even further.

Prior to Synanon, there were several examples of communities as a method for healing across England that encouraged coping from the traumatization following World War II, such as those at Northfield Hospital. The term Therapeutic Community originated here, from Thomas Main, a psychiatrist who identified "community-as-a-method" as a potential tool for preparing unwell individuals for reintegration into society. Additionally, the method had been applied to mental illness and adolescent corrections in earlier contexts as well. All of these early communities shared the common belief that wellness can be achieved through community.

Before closing its membership, Synanon had graduated the individuals who would spread the therapeutic community across the United States, and eventually the world. By training their clientele for active roles in the organization, Synanon had empowered their patients to share their model and training wherever they went. The program's hierarchical structure allowed patients to develop their skills as they gained responsibility, and ultimately became staff of the community. Much of the mystery of today's TC has been attributed to these origins of the model. The emphasis on a drug-free, hierarchical structure, and the hiring and training of former participants meant that many of the off-shoots of Synanon did not have professional staff or written methodologies. Rather, the new communities were developed according to a founder who had experienced a community firsthand.

After Synanon, the therapeutic community model started to spread across the United States until the creation of the next premier community, New York's Daytop Village in 1963. Founded by Dr. Daniel Casriel & Monsignor William B O'Brien, Daytop implored a more professional and medical approach to the therapeutic community. The movement from Therapeutic Communities across the Atlantic was largely done in partnership with Daytop Village and is responsible for much of the difference between American and European Therapeutic Communities.

(See Figure 1)

Throughout the 1970s, Europe saw therapeutic communities founded in Portsmouth, London, the Netherlands, Sweden, Ireland, and Belgium. As Daytop Germany and CEIS in Rome were directly partnered with Daytop New York, most of these first European TCs had their roots from parent organizations. By the 1980's TCs had started to spread to the remaining parts of Europe as well. Finally, throughout the 1990's and into the 2000's, the TC spread from Italy and Spain to Latin America, and from North America to Australia and Asia.

The Modern TC is most easily characterized by its emphasis on specialization. In the early 2000's financing had been reduced, due in part to results based funding initiatives and global financial downturn. Additionally, the opioid epidemic has witnessed a more sympathetic view of addiction that encourages greater treatment availability. TCs' efforts to specialize in the modern era encourage more partnership-based funding, allows for better response to their specific clientele's needs, and creates a niche in individual communities. This has increasingly become the industry standard throughout the 2010's. Moving forward, we anticipate that this trend will continue as communities try to accommodate their clients and generate the best possible results.

3. Oversight Committees

Following the quick expansion of TCs across the globe, a corresponding desire for additional resources helped create regional federations for therapeutic communities. In order of inception, the following Committees were formed: the Therapeutic Communities of America(TCA), The European Federation of Therapeutic Communities(EFTC), Australasian Therapeutic Communities Association(ATCA), the Federacion Latino Americana de Comunidades Terapeuticas(FLACT), and the Federation of Therapeutic Communities in Asia(FTCA). Additionally, there is a World Federation of Therapeutic Communities (WFTC) that was developed by experts across 26 countries and sought to provide overarching principles to the TC through an international declaration title the "Declaration of Mallorca." This became an official document of the United Nations Commission on Narcotic Drugs, officially recognizing the treatment method. Many of these oversight organizations operate as facilitators

for information exchange between communities by hosting conferences highlighting innovative organizations and their individual approaches. While the Declaration of Mallorca calls for some consistency of TC principles, the community continues to understand the need for adaptation to the target population. Many TCs now operate inside of prisons, within a religious context, or with the addition of medically-assisted treatment. As such, the principles for TCs as defined by the Declaration of Mallorca are broadly defined.

Unfortunately, the Treatment Communities of America (TCA) has less resources for individual TCs when compared to the other oversight committees, namely the ATCA. As a result, the most up to date resources are still unavailable within a centralized organization. However, an opportunity to remain up to date with current practices could be to find an oversight committee that has similar views of treatment and to utilize their resources. Similarly, and in keeping with the tradition of the TC model, TCs can compare practices between themselves. Given the difficult financial landscape during the COVID-19 pandemic, TCs may create further resilience by sharing strategies for survival.

4. Internal & External Observation

In lieu of the COVID-19 Pandemic, this report lacked the originally planned interviews with staff and competitors in the Pittsburgh TC market. To address these gaps, we observed perceived organizational status through the 2 toolkits for internal reflection and our own observations. Firstly, we compared the practices of XXX with the Australian Therapeutic Communities Administration (ATCA) by means of an ATCA questionnaire. Along with annual presentations from the most innovative TC organizations, the ATCA provides a wealth of resources for TCs. Included is an up to date set of standards for the TC. By examining the differences in XXX's approach with the ATCA Standards, there is an opportunity to recognize opportunities for adaptation. Secondly, two members of the XXX staff conducted an internal reflection using SWOT analysis. SWOT analysis provides an ideal opportunity to observe perceived internal practices while also preparing for external changes. Additionally, we offer our own observations comparing the practices of TC organizations observed in the literature review.

One observation that is identified in both the internal methodologies as well as our own observation is the desire for a culture of learning. TCs must balance the tradition of promoting from within and the trend towards professionalization. A Strategic Plan for Human Resources can be used to clarify roles and promote the culture of learning across the organization. A culture of learning would prioritize professional development and organizational knowledge. One potential area for internal growth would be to promote cross-departmental and cross-service learning. TC operations are difficult to both explain and understand - even internally. Therefore, a staff members' role and contributions may not be clearly defined or understood by others. A better understanding of the roles across departments may strengthen the XXX approach by promoting a team-oriented approach and encouraging a culture of learning.

Another takeaway from the observations is the need for a more systematic approach to Aftercare and to reintegration post-treatment. One major difficulty in Aftercare is its definition. Some see Aftercare as the end stage of residential treatment, some see it as outpatient therapy, and others the final stages of habitation. What is necessary is a cohesive approach to Aftercare that ensures a systematic transition, continued involvement, and support.

Lastly, from XXX's SWOT analysis, we identify a couple of *Strengths* that had not been stated in the initial response. XXX is a non-profit that operates on a self-sustained budget. While this is not favorable, it does promote sustainability. Growth may be subject to donors, but

operation may occur without reliance on a single entity. There are large vulnerabilities facing XXX, but they are largely deterministic of their own future. Similarly, XXX's greatest asset may be its own property. Two large homes in the heart of a rapidly developing area may better be viewed as a financial asset than a home. The service XXX provides is not neighborhood specific, and this leverage may provide XXX resiliency in financially difficult times.

5. Successful Treatment Predictors

XXX Inc. is dedicated to the best care for their clients. The goal of treatment for TCs is for participants to return to a stable and pro-social life. As such, it is essential that treatment is informed by the best techniques and utilizes the tools that encourage a pro-social life after treatment. While the TC literature remains limited compared to other treatment methods, there are numerous studies that can be considered evidence-backed in their examination of the practice. We have summarized many of the largest and most influential studies in (See Figures 3&4). However, it is important to address all the indicators for a successful outcome including myths, demographics, and treatment processes. Outcomes are most frequently measured using employment, housing, time without substance use, and judicial involvement.

Firstly, the largest misconception regarding substance use assumes that the chemical component is the largest determinant of outcome. However, nearly every study we have examined finds a negligible difference in treatment outcome when looking at the primary substance used. This is an important misconception to address as it gives merit to the TC and trauma-informed approaches. By understanding that substance use can be attributed to factors outside of a substance's chemical properties, it is easier to understand the influence of personal history and other factors.

Secondly, demographics do play a part in TC outcome success rates. There are mixed results in the importance of marriage, divorce, and children. For example, many individuals choose to leave treatment to reunite with their loved ones. Sometimes their loved ones support their new lifestyle and sometimes they are a trigger. The impact of loved ones is largely dependent on the types of relationships that exist. However, a consistent demographic influence is age, and the idea of "aging-out" of substance use. Success rates are generally higher for older aged individuals, as indicated through multiple studies.

Lastly, the treatment factors influencing change are imperative to understand in order to better address the needs of the TC participants. Most consistently, the largest factor determining program success rates is the length of treatment. This finding, along with the tendency of TCs to have higher dropout rates than other modalities, has led to a focus on the causes of retention. Oftentimes, treatment length is determined by insurance payments. However, beyond this, there are personal relationship factors, the formation of relationships inside of treatment, and numerous other influences. One development from these examinations has been the temporary use of medically-assisted treatment with the aims of keeping individuals in treatment longer. Beyond length of stay, consistent indicators of successful treatment (in no specific order) are:

1. Self-Selection
 - a. Self-selection into care is highly correlated with better outcomes across all measures. Important to note is that this self-selection includes primary treatment or voluntary aftercare.
2. After-Care Completion
 - a. Continuing care through aftercare helps in the transition from the structure of TC life to the freedom and temptation after treatment.

3. Post-treatment Employment
 - a. Unsurprisingly, steady income and employment contribute to a pro-social lifestyle after treatment.
4. Recognition & Receipt of Graduation Certificate
 - a. In many studies, when treatment length was shortened for circumstances outside of the participants control (i.e. Insurance), the recognition of their dedication to the program showed positive results on their outcomes. The impacts are amplified after completing the program at its intended length.
5. Previous TC Enrollment
 - a. TC's acknowledge relapse as an opportunity for learning and an opportunity for growth. Similarly, TC dropouts exhibit better results given a second opportunity at treatment.
6. Self-Perception of Care
 - a. Similar to the lessons learned by self-selection, many individuals exhibit better results when they perceive care to have high potential in generating personal change.
7. Impact of Push-ups & Pull-ups
 - a. Pushups & Pullups are feedback within cohorts aimed at shaping individuals' behaviors. However, the impacts of push-ups and pull-ups are not entirely positive. One study found that giving feedback results in better outcomes for the giver and worse outcomes for the receiver. Thus, feedback must encourage participation without discouraging one's self-perception of their own care.

6. Summary:

The TC field is incredibly complex and unique in its development and its behaviors. In the past decade, the industry has trended toward a more specialized and professional approach to Substance Use Treatment while balancing a tradition of community-as-a-method approach. The community has seen decreasing funds relative to an increasing public interest in substance use disorders. Furthermore, the TC community is moving towards a results-based format, which emphasizes patient outcome. Therefore, our recommendations reflect this changing landscape.

1. Emphasize self-learning by monitoring clients during and after program completion.
 - a. In the era of results-based approaches, begin to self-evaluate your own programs and test new methods. Understand how the treatment has impacted lives after treatment, and why individuals leave treatment early.
2. Understand and prioritize the indicators for client success.
 - a. Patient length of stay is the most influential indicator for patient outcome. Pursue creative methods to encourage retention and extend stays.
 - b. Understand the indicators of success and help your clients achieve success by supporting these areas.
3. Anticipate future trends and funding difficulties and set a clear direction for the type of TC you wish to become.
 - a. Set an organizational path that caters to your population. Embrace this specialized population and find partners within the sect.
4. Develop a clear and succinct marketing message to the public that describes your interventions and purpose.

- a. Remove the mystery surrounding the TC whenever possible. Find a way to convey your practice to the public in a way that is simple and consistent and removes stigma.
- 5. Find a partner to compare practices with.
 - a. Create resilience for future changes by finding organizational partners. Share and receive knowledge about successes and failures.
- 6. Develop a Strategic HR Plan to Hire and Share Information between Departments.
 - a. Create a strategic HR plan. Embrace a culture of learning by promoting knowledge sharing between the different treatment services of XXX Inc and clarify staff roles.

A1: Appendix:

Possible Funding Opportunities

- 1. PATH Grant- Substance Abuse and Mental Health Services Administration
 - a. The Projects for Assistance in Transition from Homelessness (PATH) grant is a block grant given to all 50 states for distribution to organizations that help clients with mental health problems that are experiencing homelessness. The provider of the grant, SAMSHA is looking for

clients that provide one or more of the following services: screening and diagnostic treatment, habilitation and rehabilitation, community mental health, substance use disorders treatment, referrals for primary health care, job training, educational services, and housing services as specified in Section 522(b)(10) of the Public Health Service Act. The Pennsylvania Department of Drug and Alcohol Programs is the distributor of the funds for the state. The contact for Pennsylvania's PATH grant funds is Michelle Baxter, 717 346 0752, mibaxter@pa.gov. We believe this is a perfect grant opportunity for XXX as many similar non-profit organizations within the state have received this funding, this funding could be used for a potential ¾ way housing as well. Unfortunately, the time to apply this funding has passed, but XXX should be on the lookout for the grant in the coming years to become a PATH provider.

2. Fisa Foundation

Rationale: The Fisa Foundation's subjects of interests for their funding are abuse prevention, health, mental health care, and human services. Their top three funding sectors are human services, health, and public safety. The Fisa Foundation is located in Pittsburgh, PA and its geographic focus is Pennsylvania, Pittsburgh, and Somalia. Their grant amounts range from <\$4k to >\$500k with most of their recent grants averaging \$50,000 given.

3. The Grable Foundation

Rationale: The Grable Foundation is located in Pittsburgh, PA and its main region of funding is Pennsylvania. The top three areas of funding for the Grable Foundation are education, arts and culture, and human services. The support strategies of The Grable Foundation that fit XXX are continuing support and general support. The foundation generally gives out grants in the \$10k to \$100k range. The Grable Foundation is a good fit for XXX as in 2016 they gave a grant for \$30,000 to the saltworks theater for addiction services.

4. W. Clark Hagan Trust

Rationale: The W. Clark Hagan Trust is located in Pittsburgh, PA and gives primarily to Pittsburgh. The population groups that the trust supports include those with substance use disorder, and the subjects that they have funding interests in are substance use services, health, human services and mental health care. However substance use is their main area of focus. The majority of their grants range in the \$10k-\$20k range, and they do general support funding.

5. The Heinz Endowments

Rationale: The Heinz Endowments are located in Pittsburgh, Pa, their main area of funding is their local community of Pittsburgh. The top three areas of funding are community improvement, education, and environmental services, but their fourth area of funding is human services and their sixth is health. The Heinz Endowments population focus is dropouts, economically disadvantaged people, girls, low-income, and young adults. Their strategies for giving include general support, and system or operational support. The Heinz Endowments is a potential funder for XXX because they have in the past funded addiction services. In 2019 they gave a \$50k grant to NurturePA for substance use services and in 2016 they gave a \$1 million grant to Action-Housing Inc. for housing development. This shows they will support substance use services and infrastructure. Also, the Heinz Endowments give large grants with the majority being in the \$25k to \$100k range, but they have been known to give in the \$500k to 1 million range.

6. Highmark Foundation

Rationale: The Highmark Foundation is located in Pittsburgh, Pa and gives primarily in southwestern pa, and West Virginia. The three main focuses of funding for the foundation are health, education, and human services. The subjects of interest for the Highmark foundation generally vary but most remain in the healthcare industry. The ones pertinent to this project are

health, human services, in-patient medical care, mental health care, out-patient medical care, and public health. The people they are interested in supporting through funding are low-income people, people with disabilities, and economically disadvantaged people. The Highmark foundation has in the past funded addiction services at Central Outreach Resource and Referral Center, this grant total \$6k and was in 2016. Their support strategies include equal access, equipment, general support, and operational improvements. The majority of Grants given by the Highmark Foundation are within the \$50-100k range.

7. The Pittsburgh Foundation

Rationale: The Pittsburgh Foundation has been a grant funder for XXX in the past. The Pittsburgh Foundation is located in Pittsburgh and has a primary focus of supporting and funding the local Pittsburgh Community. The majority of funding by The Pittsburgh Foundation average less than \$5k dollars per grant, but in the past has given grants in upwards of \$30k to substance use services. The top three areas of focus for giving for the Pittsburgh Foundation are education, human services, and community development. However, healthcare is a close fourth place in what is being funded for the Pittsburgh Foundation.

8. United Way of Southwestern Pennsylvania

Rationale: The United Way of Southwestern Pennsylvania is located in Pittsburgh, PA and primarily gives in Allegheny County. The top three areas that are being funded by the United Way are human services, philanthropy, and health. The subject areas of focus for the funder are human services, in-patient medical care, mental healthcare, non-profits, and special population support. The funder has previously funded rehabilitation services in 2018 with a grant of \$16,468. The Organization's purpose and activities are to increase funding and support quality health and human service agencies in Allegheny County. The majority of their grants are in the \$5-25k range, but they have given grants of over \$100k.

9. The Cooper-Siegel Family Foundation

Rationale: The Cooper-Siegel Family Foundation is located in Pittsburgh; Pa and they primarily give their funding to organizations within Pittsburgh. Their top three areas of giving are health, human services, and philanthropy. The majority of their grants are within the \$5-25k range. As recent as 2016 they have given a gift of \$5k to an organization for rehabilitation. They focus their gives toward general support and focus giving to human services, economically disadvantaged people, and low-income individuals.

10. Federal Investors Foundation Inc.

Rationale: This foundation is based out of Pittsburgh, PA and primarily focuses its giving to the local community of Pittsburgh. The top three areas of funding that this organization primarily has focused on is health, community development and education. The foundation's support strategies include capital and infrastructure. The foundation has given \$12,500 to health-based organizations in 2018, the majority of their giving is in the >\$5k range but a close second is the \$10-25k-dollar range.

11. John R. McCune Charitable Trust

Rationale: The John R. McCune Charitable Trust is located in Pittsburgh, PA and primarily focuses their giving in the Southwestern region of Pennsylvania. The top three areas of focus that the organization gives to are education, human services, and health. The organization funds capital and infrastructure, as well as, capital campaigns as part of its strategic support strategies. The majority of its grants are usually totaling \$20k and the majority of its grants are in the \$10-25k range. The purpose of the trust is to support education, health, and social services.

A2. Integrated Treatment for Dual Diagnosis

Substance abuse disorders are the most common comorbid disorder among adults with severe mental health illness. These types of diagnosis are classified as dual diagnosis, which in this case refers to the co-occurrence of substance abuse disorder along with severe mental illness. An underlying cause of substance abuse disorder is the presence of an undiagnosed mental illness, but this does not mean that one cannot be present without the other. Rather this is saying that co-occurrence or a dual diagnosis is common as research has shown that 50% of people diagnosed with a severe mental illness have also been affected by substance abuse or diagnosed

with a substance abuse disorder.¹ This is one of the three findings that most of the research done on dual diagnosis of substance abuse disorders and severe mental illness has consistently agreed upon. The other two are as follows: 1. Dual diagnosis is often associated with worse outcomes for patients. These include but are not limited to high rates of relapse, hospitalization, violence, incarceration, homelessness, and serious medical disorders such as HIV and Hepatitis. 2. Patient care is often fragmented into either substance abuse treatment or mental health treatment. The United States' healthcare system often does not treat these individuals with effective care, as they are not treating the illnesses in full¹. There are programs in the United States that treat with dual diagnosis in mind, but there has been constant failure at a state and federal level to support this type of care. This can be attributed to a number of factors including little attention to quick policy changes arising from bureaucratic nature or the fact that most of the literature on dual diagnosis treatment is inconsistent¹. The programs that are succeeding in their approach to treating dually-diagnosed individuals share the following components¹:

1. Staged interventions- The creation of non-linear stage of treatment
2. Assertive Outreach- Consistent outreach to the client and their support systems
3. Motivational Interventions- Helping the client realize their own goals and educating the clients that using drugs or alcohol to cope with their illness is detrimental to their success
4. Counseling- Cognitive behavioral approaches to counseling that can take many forms such as group, individual, or family counseling
5. Social Support interventions- Strengthening the immediate social support network of client to help modify their behavior
6. Long-term perspective- Recognizing that recovery is a long-term process that may take months or years in the community
7. Comprehensiveness- In order to lead a successful and satisfied life client must understand that this requires changing many aspects of one's life

Even though there is strong support for dual diagnosis treatment the barriers to such programs still exist in many treatment facilities. The barriers can be many, but are broken down into four categories: policy, clinical, program, and consumer and family¹. Policy barriers have to do with the governmental infrastructure and organization of cabinet level agencies that oversee the mental health and substance abuse regulation. Clinical barriers are mostly at the ground level with treatment facilities not accepting the use of dual diagnosis treatment practices or a lack of education and training of the clinical staff to truly implement the treatment practices. Program barriers much like clinical barriers exist at the ground level with administrations not being trained or educated into the implementation of dual diagnosis treatment. Family and consumer barriers have to do with the lack of general education of dual diagnosis, clients that are not familiar or have not been treated in a dual diagnosis setting are often not willing to participate in these programs.

In conclusion, dual diagnosis treatment has been proven effective in providing a system that better understands and treats the underlying illnesses and problems that lead to substance abuse. The research is inconsistent because of the lack of implementation by the field. The field for dual diagnosis is growing through and will continue to be a system that acknowledges substance abuse disorder and mental illness as confounding illness rather than the current fragmented treatment plans that we see in the United States especially at the larger governmental levels. The best strategies for implementation of dual diagnosis treatment is education, this includes education of policy advisers, program creators, clinical treatment staff and the clients

themselves. The further implementation of dual diagnosis treatment will have an impact on the treatment of the people that need it the most.

1. Drake, R. E., Essock, S. M., Carey, K. B., Minkhoff, K., Kola, L., Lynde, D., . . . Richards, L. (2001). Implementing Dual Diagnosis Services for Clients with Severe Mental Illness. *Psychiatric Services*, 469-476.

A3: Therapeutic Community Literature Review

Introduction

The following literature review provides an introduction to the development of the Therapeutic Community as a means of treating substance use disorder. Additionally, the literature review presents the evidence supporting TCs as a cost-effective and results-based practice. We find that the literature readily supports both of these claims reinforcing TCs as a legitimate and effective form of treatment. Lastly, when possible, we seek to identify predictors of treatment success.

Resources

The literature explored in this review combines publications from the United States and globally that attempt to give a comprehensive view of the Therapeutic Community. The sources identified thus far include publications from the journal of therapeutic communities and other academic and medical journals, books that detail the history and origin of the topic, presentations from governing bodies, and sources made available from our client, XXX, Inc..

Defining the TC

Therapeutic Communities (TCs) are a “Community-as-a-method” approach to addiction recovery that has spread across the world as a solution to personal wellness and addiction recovery. As the method has been individually adapted by numerous countries and institutions, the model itself has become increasingly difficult to define. For the purposes of this paper and moving forward we will explore therapeutic communities in a broader sense to be more inclusive to ideas that are being explored in the “community-as-a-method” model. Therefore, the TC is a treatment modality that addresses addiction via mechanisms of action that differ from Medically Assisted Treatment (MAT) and other common interventions currently available. TCs offer a residentially based treatment program with a relatively long length of stay (although the LOS has varied greatly over their histories and between customized approaches) (Avery, 2019). TCs approach change of cognitive thinking through a social learning approach that includes peer led meetings and a community largely operated by the residents. Additionally, TCs work on habilitation and occupational training in coherence with their ultimate aim, the successful reintegration of their clientele into society (Avery, 2019). Once again, therapeutic communities have continued to develop and diversify, and as such, many TCs vary drastically in their individual approaches.

The First Therapeutic Communities

The Therapeutic Community model is often attributed to the 1958 Synanon Village and founder Charles Dederich, which began as a community for those suffering from addiction to prepare themselves for reintegration back into society. However, in the 1970’s, Charles Dederich used his authoritarian grip over the community to close the community and remove reintegration as the primary goal, leading to the transformation into a cult-style community (Avery, 2019). However, while this infamous failure has led to many misperceptions about TCs, the TC’s origins trace back even further.

Prior to Synanon, there were several examples of communities as a method for healing across England that encouraged coping from the traumatization of following World War II, such as those at Northfield Hospital. The term Therapeutic Community originated here, from Thomas Main, a psychiatrist who identified “community as a method” as a potential tool for preparing unwell individuals for reintegration into society. Additionally, the method had been applied to mental illness and adolescent corrections in earlier contexts as well. All of these early communities shared the common belief that wellness can be achieved through community (Avery, 2019).

Therapeutic Communities in the US

Before closing its membership, Synanon had graduated the individuals who would spread the therapeutic community across the United States, and eventually the world. By training their clientele for active roles in the organization, Synanon had empowered their patients to share their model and training wherever they went. The program’s hierarchical structure allowed patients to develop their skills as they gained responsibility, and ultimately became staff of the community. Much of the mystery of today’s TC has been attributed to these origins of the model. The emphasis on a drug-free, hierarchical structure, and the hiring and training of former participants

meant that many of the off-shoots of Synanon did not have professional staff or written methodologies. Rather, the new communities were run by a creator who had experienced a community firsthand (Avery, 2019).

After Synanon, the therapeutic community model started to spread across the United States until the creation of the next premier community, New York's Daytop Village in 1963. Founded by Dr. Daniel Casriel & Monsignor William B O'Brien, Daytop implored a more professional and medical approach to the therapeutic community. The movement from therapeutic communities across the Atlantic was largely done in partnership with Daytop Village and is responsible for much of the difference between American and European Therapeutic Communities (Vanderplasschen, 2014).

(See Figure 2)

Throughout the 1970s, Europe saw therapeutic communities founded in Portsmouth, London, the Netherlands, Sweden, Ireland, and Belgium. As Daytop Germany and CEIS in Rome were directly partnered with Daytop New York, most of these first European TCs had their roots from parent organizations. By the 1980's TCs had started to spread to the remaining parts of Europe as well. Finally, throughout the 1990's and into the 2000's, the TC spread from Italy and Spain to Latin America, and from North America to Australia and Asia (Broekaerta, 2006).

Formation of Oversight Committees

Following the quick expansion of TCs across the globe, a corresponding desire for resources helped create regional federations of therapeutic communities. In order of inception, they were: the Therapeutic Communities of America(TCA), The European Federation of Therapeutic Communities(EFTC), Australasian Therapeutic Communities Association(ATCA), the Federacion Latino Americana de Comunidades Terapeuticas(FLACT), and the Federation of Therapeutic Communities in Asia(FTCA). Additionally, there is a World Federation of Therapeutic Communities (WFTC) that was developed by experts across 26 countries and sought to provide overarching principles to the TC through an international declaration titled the "Declaration of Mallorca." This became an official document of the United Nations Commission on Narcotic Drugs. Many of these oversight organizations operate as facilitators for information exchange between communities by hosting conferences highlighting innovative organizations and their individual approaches. While the Declaration of Mallorca calls for some consistency of TC principles, the community continues to understand the need for adaptation to the target population. Many TCs now operate inside of prisons, within a religious context, or with the addition of medically assisted treatment. As such, the principles for TCs as defined by the Declaration of Mallorca are broadly defined (WFTC, 2013).

Why there isn't much literature for TCs

As mentioned previously, the TC model was created as an alternative to medical treatment for addiction, which utilized community members rather than medical professionals. As a result, the field never developed along results-based outcomes that are familiar in other medical aspects. However, more recently, TCs have begun to employ more professional staff with university training that bring higher results-based approaches. The Declaration of Mallorca specifically denotes the importance of research as a means of developing the Therapeutic community, as well as outlining a responsibility for organizations to track their patients through aftercare. This explains both the historical lack of results-based structure and the present movement towards pay for value (Gosling, 2016).

Evolution of the TC

The complexity of the graphic below demonstrates the numerous developments across therapeutic communities world-wide. Therapeutic communities are designed to be peer-driven and respond to the community. Therefore, unsurprisingly, the communities transform along ideological differences and the backgrounds of both participants and founders. Of the following sects of TCs, a growing literature is appearing on communities with dually-diagnosed, adolescents, prison-based TCs, female oriented programs, and medically integrated communities. (See Figure 2)

Proof and Effectiveness

Difficulties with TC Studies

The effectiveness of TCs is very difficult to prove for many reasons. Firstly, from a methodological standpoint, the golden standard would be to conduct an experiment including a random control group, that is difficult to ethically compare. As a result, critics have deemed TCs not results-based, while proponents have defended alternative methods investigating effective outcomes from therapeutic communities. Therefore, most of the literature of TCs are created using one group statistical models as well as qualitative methodologies to follow-up and evaluate clients at different periods post-treatment along a number of different indicators (De Leon, 2015).

Secondly, these studies examine a number of different input and output indicators for client success. Inputs have examined length of treatment, experiencing program graduation, experience of pull-ups & encounter groups, previous traumatic experiences and ACE scores, type of substance uses, and others. Outputs have explored harm reductions, to substance abstinence, relapse, recidivism, and decreasing frequency of usage. Success is relative and through these mixes of indicators, it is difficult to find consistent results. (Evident in Figures 3&4)

Thirdly, important to note in the examination of these findings, is that Therapeutic Communities have been noted for their ability to treat “Treatment Resistant” populations. This term does not refer to a person’s willingness to participate in treatment, but rather confounding factors influencing their addiction. This is characterized by individuals who may have had failed attempts with treatment before, did not respond well to Medically Assisted Treatment (MAT), or have had trauma that needs to be addressed prior to achieving abstinence. For example, Kast (2019) says that this difference in populations could be represented in the difference between chasing a high versus escaping a low. Treatment resistant populations have often experienced other social, financial, or judicial difficulties. As a result, it could be argued that the implications for TCs underestimate their potential when compared to results from other addiction treatment services.

Lastly, another factor that could lead to inconsistent results for TC effectiveness is the individualization of each TC. Because TCs do not have strict and consistent guidelines, differences between organizations can vary greatly. Therefore, a study of a TC may not be generalizable. Additionally, while treatment resistant populations were mentioned, some of these studies examine subgroups of treatment resistant populations specifically and may have different results. Essentially, there cannot be a consensus statistic on the effectiveness of TCs because of the variance in type, structure, management, practice, and population served. However, the chart below demonstrates that even without a consensus for overall significance of a TC, the community can still be viewed as results based. TCs consistently demonstrate high levels of improvements along different dimensions measuring financial, social, physical, and mental well-being, as well as drug use. Especially when compared to no treatment or treatment as usual.

Figures 3 & 4 demonstrate a large portion of the literature showing the effectiveness of Therapeutic Communities. Much of the chart was included in George de Leon's article: "Is the Therapeutic Community an Evidence Based Treatment? : What the Evidence Says." (2010). However, our analysis has included further studies that show the use of TCs in a wider array of contexts.

What Predicts Success within a TC

Perhaps the most important takeaways from this paper are the implications of what does and does not predict success within a TC. Consistently, longer length of stay, program graduation, participation in aftercare, self-perception about care, previous TC enrollment, posttreatment employment, and older age are correlated with better treatment success in the long-run. While MAT is being integrated into the TC space in some organizations, Stubbley (2017) has argued that methadone (and similar alternatives) should only be used as a means of retention for clients and not a primary care device. If length of stay is the primary predictor of success, retention should be the focus of TCs. Additionally, milestones like program graduation are strong predictors of success post-graduation. The program design understands that promotion of the individual through a hierarchy of responsibility is important for participants to recognize their own growth. Meanwhile, there is clear evidence that an individual's desire to participate is a predictor of their success. Previous enrollment, voluntary extensions of length of stay or participation in aftercare, and self-perception about one's own ability to succeed is strongly correlated with better success rates. Therefore: "Addressing clients' perceptions that they can succeed appears to be an important clinical implication of these data"(Darke, 2011).

Other less conclusive studies have examined the role of external networks in the treatment process. For example, participants with partners outside of care or who have experienced divorce may experience different approaches to care. Specifically, early dropout for individuals as a means of reuniting with their partners has been witnessed (Best, 2016).

Lastly, a study by Warren looked at the implications of pull-ups and push-ups in group feedback as a predictor of success as well. Those who sent more critical feedback (pull-ups) to peers were more likely to graduate while those who received more critical feedback were less likely to graduate, unless they were actively reciprocating feedback. While this may be unsurprising - that those who are more consistently breaking community norms are less likely to thrive in the community - it does give insight into the value of peer to peer relationships (Warren, 2013).

On the contrary, a couple of factors that many people believe to be predictors of graduation include psychopathology and primary substance use. However, these have been found not to correlate with early dropout. This is an important distinction and speaks to the usefulness of TCs across multiple substances and mental illnesses. Despite the type of addiction or impairment, most individuals respond to Therapeutic Community care in a similar manner. (Darke, 2011)

Conclusion

In summary, the literature has proven that TCs are a capable method of treating addiction and mental health issues in the United States and across the world. While there are inconsistent measures of success, the general understanding is that TCs have exhibited long-term beneficial impacts on personal livelihood over a variety of different indicators. Furthermore, these results usually are exhibited in a treatment resistant population for which alternative methods have failed to produce meaningful change. Lastly, the primary indicator for successful treatment is a longer length of stay, and it should be the primary goal of TCs to avoid early dropout of their clientele. While the results for TCs may vary greatly between community types, the World

Federation of Therapeutic Communities has called for organizations to be proactive in their own internal research to identify the needs of their own target population.

A4: Timeline-Graphic

The purpose of this graphic is to show the ever-evolving nature of the therapeutic community industry and to draw insights for the future. The timeline begins with the predecessor to the TC: the community as a method treatment model, which began after the second world war. Doctors began to observe the potential for using community as a method of treating PTSD, addiction,

trauma, and mental health. But these observations and institutions were unrelated. Rather, they were born out of the movements like the child guidance movement and the growing need to address PTSD, before finally reaching addiction. Around this time and the foundation of the Northfield Mental Hospital, the term Therapeutic Community was coined which would encompass all of these communities. This leads us to the second phase of the Therapeutic Community industry which spans from around the 1960s to 2000s.

This second phase characterizes the TC model that we know today and shows its spread around the world. TCs first started spreading in the United States in the 1960s and eventually to western Europe in the 1970s. This is also when we see the creation of XXX as a therapeutic community in Pittsburgh. The spread of the traditional TC model continues into the 80s with eastern Europe, the 90s with Spain, Asia, and Italy, and finally into South and Latin America in the early 2000s. During the traditional TC phase, we see the application successes across multiple industries, specifically in Prison Based TCs. This recognized potential leads to the current phase of the TC industry, which we have named the Modern Therapeutic Community.

The Modern TC is most characterized by its emphasis on specialization. At the beginning of this period, financing had been reduced due in part to results based funding initiatives and global financial downturn. Also, the opioid epidemic has witnessed a more sympathetic view of addiction that encourages greater treatment availability. TCs' efforts to specialize in the modern TC era encourage more funding, respond to their clientele's needs, and create a niche in individual communities. This has increasingly become the industry standard throughout the 2010's. Moving forward, we anticipate that this trend will continue as communities try to accommodate their clients and generate the best possible results.

Years:	<1940	1950	1960	1970	1980	1990	2000	2010	2020 & Beyond
TC Eras:	Community as a Method		Expansion of Therapeutic Communities				2000	Therapeutic	Specialization
Milestones	Term Therapeutic Community Originated	1958- First Addiction Based TC		1971- Alpha House Founded			Programs shorten to 12-9 months	Communities lose funding amidst results based payments & global recession	
TC Expansion	Community Based Treatment begins being applied as a pilot project to many social ailments	US pilot programs begin	Widespread use in the United States	Western Europe	Eastern Europe	Spain, Italy, Asia	South America	Opioid Epidemic	TCs continue to specialize. Niche TCs become the industry standard
Adolescents	Child Guidance Movement Begins - Hoping to address child delinquency	Child Guidance Clinics shift focus from inner-city areas to middle-class children							Specialized Adolescent TCs cater to the needs for coping with family issues, reducing substance use & criminal behaviors, and developing Life Skills
	Juvenile Courts Created	Field moves away from community approach as they emphasize recognizing internal factors rather than external							
	Child Guidance Clinics Introduced								
PTSD & Mental Health	Northfield Mental Hospital started to treat WWII soldiers	1964 - Community Mental Health Services Act provides funds for mentally ill		The National Alliance for the Mentally Ill founded to provide support, education, advocacy, and research services for people with serious psychiatric illnesses		Modified TCs Begin Catering to Dually-Diagnosed Individuals			Programs like the Program for Traumatic Stress Recovery join TCs and Trauma Models to encourage recovery
Addiction	1938 - 12 Step AA begins	Synanon Village	Methadone Maintenance Treatment Introduced				2002- Buprenorphine Approved by FDA	Increasing Opioid Deaths Nationwide	Addiction Treatment Communities begin to specialize to better address the needs of patients including: Medically Assisted, Dually-Diagnosed, Female, Prisoners, Faith-based, etc.
Prison Based			First Prison Based Application - Akelepton	Pilot Prison TC Programs		1990 Premier Anity Prison TC Introduced (200 total Prison TC enrollees)	2000 - 10,000 total Prison TC enrollees		Prison Based TCs boast the highest success rates in reducing recidivism and drug use which encourages it's expanded applications
Years:	<1940	1950	1960	1970	1980	1990	2000	2010	2020 & Beyond

A5: Figures

Figure 1 - (Broekaerta, 2006)

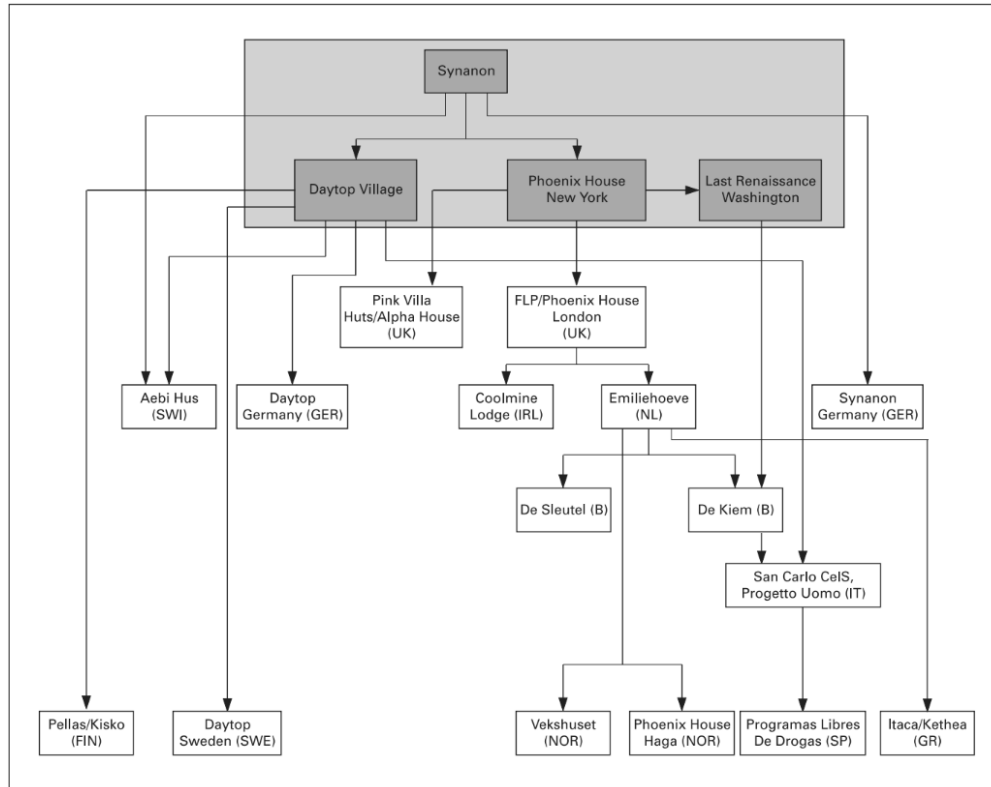


Figure 2 - (Perfas, 2019)

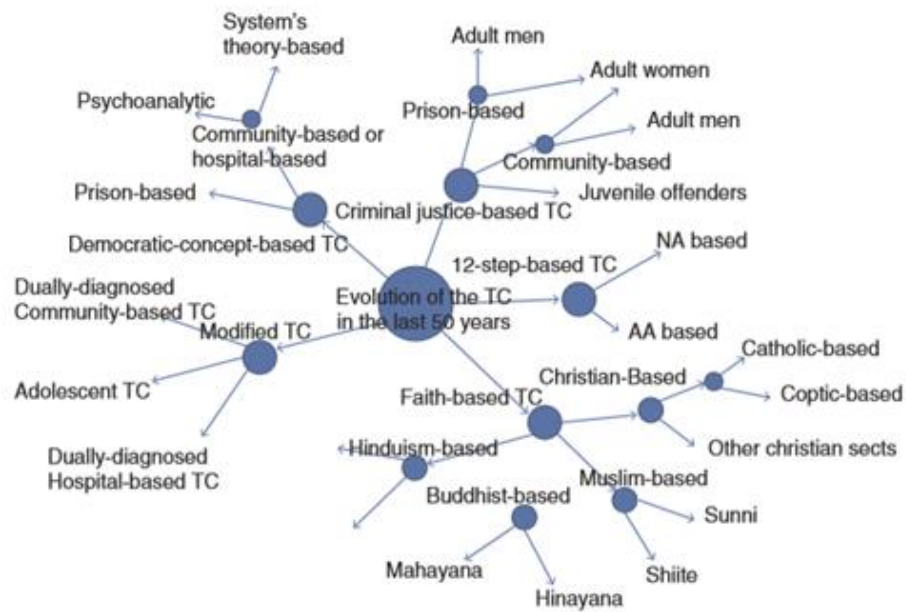


Fig. 3.2 Hybridization of the TC through cultural diffusion

Figures 3&4 - De Leon (2010) et. Al

<i>Meta Analysis Studies</i>				
Author	Article	Year	Description	Findings
Lees, Manning & Rawlings	A culture of Enquiry: Research Evidence and the Therapeutic Community	2004	Reviewed 29 studies including 8 randomised control trials, 11 Addiction TCS+ 18 TCS for personality disorder.	An overall statistically significant effect for TC treatment, with an overall summary log odds ratio of -.512 (95% confidence interval from -.598 to -.426). Concluded evidence supports the comparative effectiveness of the Addiction TC in prison settings vs. the non-Addiction TC.
Mitchell, Wilson & Mackenzie	Does incarceration-based drug treatment reduce recidivism? A meta-analytic synthesis of the research	2007	Conducted a recent meta-analysis of treatment of incarcerated offenders.	Conclusion: at this point TCS have the strongest level of empirical support of any treatment aimed at this population.
Pearson & Lipton	A meta analytic review of the effectiveness of corrections based treatment for drug abuse	1999	Conducted a meta-analysis of seven prisonbased TC programmes	Six of the seven TC evaluations showed reduced recidivism to a statistically significant degree over the comparison groups
Prendergast, Podus, Chang & Urada	The effectiveness of drug abuse treatment: A meta-analysis of comparison group studies	2002	Conducted a larger meta-analysis of drug treatments in general including TCS.	These investigators found an average effect size (adjusted for methods quality) of $g = 0.25$ (eight studies), which is equivalent to about a 12% difference between (TC) treatment and comparison conditions.
Smith, Gates & Foxcroft	Therapeutic communities for substance related disorder	2006	Conducted a meta analysis involving various different comparisons.	There is little evidence that TCS offer significant benefits in comparison with other residential treatment, or that one type of TC is better than another. Prison TC may be better than prison on its own or Mental Health Treatment Programmes to prevent re-offending post-release for inmates. However, methodological limitations of the studies may have introduced bias, and firm conclusions cannot be drawn due to limitations of the existing evidence.
Springer, McNece & Arnold	Substance Treatment for Criminal Offenders: An Evidenced Based Guide for Practitioners.	2003	Overview analysis examining what interventions work in assessing and treating substance-abusing criminal offenders.	The authors commented that methodological limitations do not permit them to conclude that TCS are more successful than other practice approaches in reducing recidivism.
Wouter Vanderplasschen, Kathy Colpert, Mieke Autrique, Richard Charles	Therapeutic Communities for Addictions: A Review of Their Effectiveness from a Recovery-Oriented Perspective	2013	Overview examining additional predictors of TC success rates as well as TC retention ability.	The study demonstrates that participation in aftercare, posttreatment employment, and older age were found to be the most common predictors of abstinence and absence of rearrest for TC graduates. However, TCS also show significantly lower retention rates compared to other treatment modalities.

<i>Individual Studies</i>				
Author	Article	Year	Description	Findings
MICHAEL T. FRENCH STANLEY SACKS GEORGE DE LEON GRAHAM STANES KAREN MCKENBRICK	MODIFIED THERAPEUTIC COMMUNITY FOR MENTALLY ILL CHEMICAL ABUSERS: OUTCOMES AND COSTS	1999	Explores a Cost-Benefit Comparison between Treatment-As-Usual (TAU) to TCs demonstrating that they are approximately the same cost for treatment.	The multivariate analyses indicated that the modified TC program facilitated significant improvement in nondrug related criminal behaviors, psychological symptoms, and type of employment
Steve Pearce, Usle Scott, Gillian Attwood, Kate Saunders, Madeleine Dean, Ritz Die Ridder,	Democratic therapeutic community treatment for personality disorder: randomised controlled trial	2017	Evaluated the usefulness of a single TC model (n=70) for Personality Use Disorders	Results shown at 12 and 24 months: Significant difference in Modified Overt Aggression Scale, Client satisfaction, and self harm for TC graduates. However, insignificant difference in Social Functioning & Mental Health Measures.
Pendergast, Podus, Chang & Utrada	AMITY PRISON-BASED THERAPEUTIC COMMUNITY: 5-YEAR OUTCOMES	2004	This study observes the combination and outcomes of Primary, Secondary and Tertiary treatments in the Amity Prison Therapeutic Community.	Results show minimal statistical differences across employment or recidivism among those who were not of advanced age and those who did not complete aftercare. However, those who completed aftercare demonstrated a 40% difference in incarceration rates at 5 years versus the control group.
Petra K. Staiger, Paul Ukhtatzy, Amelia J. Lake, & Stefan Gruenert	Longitudinal Substance Use and Biopsychosocial Outcomes Following Therapeutic Community Treatment for Substance Dependence	2020	Observation of 2 separate TCs at 9 month post-treatment measures examines substance use, dependence & biopsychosocial outcome (Identified as subjective well-being, social functioning, and mental & physical Health)	At 9 months post treatment, 68% of participants had achieved 90 day drug abstinence, substance use frequency was down 50%, drug and alcohol dependence down 60%. Additionally, there was a doubling of wellbeing score and a halving of psychiatric severity scores. At 3.5 years graduates who also participated in aftercare showed a 24% increase in drug abstinence relative to the no treatment control group, and a 13% increase over those who participated and dropped the program. At 5 years, graduates also completing aftercare showed a 25% decrease in rearrest probability compared to program dropouts.
James A. Inciardi Steven S. Martin Clifford A. Butzin	Five-Year Outcomes of Therapeutic Community Treatment of Drug Involved Offenders After Release From Prison	2004	This study explores long term implications of a Prison based TC on Drug Abstinence and Rearrest.	At 3.5 years graduates who also participated in aftercare showed a 24% increase in drug abstinence relative to the no treatment control group, and a 13% increase over those who participated and dropped the program. At 5 years, graduates also completing aftercare showed a 25% decrease in rearrest probability compared to program dropouts.
Christopher J. Sullivan, Karen McKendrick, Stanley Sacks, and Steven Banks,	Modified Therapeutic Community Treatment for Offenders with MICA Disorders: Substance Use Outcomes	2007	Explores the impact of modified dual treatment TC for inmates with Mental Illness and Chemical Abuse disorders.	Substance use showed 34% less than the control group, while crime and recidivism directly related to amount of usage after release.
Sacks, Sacks, McKendrick, Banks & Stormiel	Modified TC for MICA offenders: Crime outcomes	2004	Compared a modified prison TC for inmates with cooccurring mental illness and substance abuse with those in an enriched mental health treatment in prison).	TC sample was significantly less likely to be reincarcerated and had better drug and psychological outcomes than those randomly assigned to the mental health programme.
Wexler, Melnick, Love & Peters	Three year reincarceration outcomes for Anity in-prison therapeutic community and aftercare in California	1999	Three-year reincarceration outcomes for in-prison TC and aftercare.	Significantly fewer prisoners who had gone through a TC followed by an aftercare programme had recidivated versus the comparison condition.

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